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FOCUS GASTROENTEROLOGY

17 April 2026

Dr Tamson Walpole
Mills Street Clinic

Dear Dr Walpole,

Re: Mr Nicholas Hamilton
DOB: 1976-04-28
Address: 117 Nelson Road, SOUTH MELBOURNE, VIC, 3205

Summary

- **Deep clinical and histological remission of Crohn's disease.**
- **Weaning of mesalazine with a view to continue ustekinumab monotherapy.**
- **Three-day trial of prednisolone to evaluate steroid-responsiveness of joint pain.**
- **Surveillance colonoscopy recommended in three to five years.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Nicholas Hamilton to discuss his recent procedure results and ongoing management. I am pleased to report that the histopathology from his recent colonoscopy confirms he is in a state of deep remission. The biopsies showed no evidence of active inflammatory bowel disease, with only a single, clinically insignificant patch of cells noted in one sample. His risk of a flare or future malignancy is currently very low.

Nicholas reports that his gastrointestinal symptoms remain well-controlled. Given his current stability, we discussed de-escalating his oral therapy. He is currently on ustekinumab injections, which are likely providing the vast majority of his therapeutic benefit. I have advised him to reduce his mesalazine to one tablet daily for one month before ceasing it entirely. Should he notice any return of symptoms during this weaning process, he has been instructed to revert to his original dose and contact me.

We also discussed his symptoms of bloating and flatulence following the ingestion of dairy. While these symptoms are consistent with lactose intolerance—which is common in the general

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

population and particularly in those with inflammatory bowel disease—I have advised him that this does not cause structural damage to the bowel. I have not recommended formal breath testing, as it would not alter our clinical management; he will continue to manage his dairy intake based on his symptomatic tolerance.

Impression and Plan The primary concern for Nicholas at present is persistent joint pain. It remains unclear whether this represents active immune-mediated inflammation or residual damage and altered pain signalling. To clarify this, I have provided him with a three-day trial of 25 mg prednisolone. I have asked him to monitor his joint symptoms closely and email me regarding the response. If the pain is steroid-responsive, I will coordinate with his rheumatologist, Steph, to consider whether a transition to an alternative immunosuppressive agent better suited for joint involvement is necessary. Regarding his recent colonoscopy, a benign but precancerous polyp was removed. In conjunction with his remission status, I have recommended a surveillance colonoscopy in three to five years. I have provided scripts for his ustekinumab and requested interval blood tests. I plan to review him again in six months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Helen Aharfi, Dr Stephanie Bond

Investigation Results:

MICROSCOPIC DESCRIPTION And DIAGNOSIS 1. Right colon - Mild distortion of crypt architecture. No erosion. No thickening of the collagen plate. No atypia. There is a very focal neutrophil infiltrate with some cryptitis but no crypt abscesses. There are no granulomas. 2. Left colon - Normal crypt architecture. No erosion. No inflammatory infiltrate. No thickening of the collagen plate. No atypia. 3. Sigmoid biopsy - Normal crypt architecture. No erosion. No inflammatory infiltrate. No thickening of the collagen plate. No atypia. 4. Rectal polyp - Tubular

adenoma with low grade dysplasia. 5. Rectal biopsy - A hyperplastic polyp.